Validation report - dapagliflozin population PK simulations (dapapksim)

Date: 20

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Source code: https://github.com/Robterheine/dapapksim (`app.R`, `R/pk_model.R`)

HTML version: `docs/VALIDATION_REPORT.html` (self-contained; export to PDF via browser print)

PDF version: `docs/VALIDATION_REPORT.pdf`

Status: pre-final, reviewed (see Appendix B: review log)

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1. Introduction

This report validates the population pharmacokinetic (popPK) model implemented in the Shiny app **dapapksim** - an R implementation of the NONMEM model of Penland et al. 2023, with a renal/non-renal clearance split, allometry on body weight (BWT/84), UGT1A9 maturation, and no sex effect on CL. The model is validated numerically and graphically against **three independent publications**:

1. **Penland et al. 2023** - J Clin Pharmacol 63:551-559, doi:10.1002/jcph.2196 (source popPK model; Table S3/S4; CL/F ~ 21.6 L/h).
2. **Tirucherai et al. 2016** - Diabetes Obes Metab 18:678-684, doi:10.1111/dom.12638 (single doses 2.5/5/10 mg in 24 children/adolescents aged 10-17 y with T2DM; Table 2).
3. **Jo et al. 2021 (PBPK)** - CPT Pharmacometrics Syst Pharmacol 10:108-118, doi:10.1002/psp4.12577 (verified PBPK model; prospective predictions per age group 1 month-18 years; Table 3; its own acceptance criterion 0.5-2-fold).

Reading guide. Section 4.1 is written for a clinical pharmacologist, §4.2 for a biomedical statistician. A *figure* is a graphical summary of the validation: every figure in this report ends with an explicit verdict (PASS / PASS with caution), and §3 opens with a summary table of all verdicts. Unit scaling (mg -> µg, L, ng/mL) was verified separately.

2. Methods

- **Model.** 2-compartment (ADVAN4/TRANS4), first-order absorption with 0.5 h lag, linear elimination. `CL = 5.44-MAT-(BWT/84)^0.75 + 0.177-eGFR-(BSA/2)` (eGFR in mL/min/1.73 m² from copulapop; BSA in m² from copulapop, Mosteller; BSA_ref = 2 m² assumed for the source population - see §4.3); `MAT = 0.05 + 0.95-AGE^1.9/(0.09^1.9 + AGE^1.9)` (Farhan 2024); KA/V2/V3/Q allometric on BWT/84. IIV log-normal: KA omega² = 8.52 (fixed, source), CL/F 0.1136, Vp/F 0.8229. **No residual error** - the 90% band is a prediction interval across individuals. IOV on F1 = 0% in this validation. Full mapping: `model_doc.md` (tab "Model & assumptions" in the app).
- **Simulation.** RK4, dt = 0.05 h. Penland comparison: 10 mg once daily, 7 doses (steady state). Tirucherai/Jo comparisons: **single dose** (0-48 h). Per-subject metrics: Cmax, tmax, AUC (trapezoidal; AUC0-inf = Dose/CL), CL/F, terminal t1/2.
- **Virtual populations.** Tirucherai: N = 2000 sampled around the study means (age 14.5 ± 2.0 y (10-17), weight 99.7 ± 26.4 kg, eGFR 112.9 ± 20.5, 37.5% male). Jo/PBPK: per age group one typical virtual patient (median weight for age; age-based

eGFR, same formula as the app fallback). Seed 20261105.

- **Comparison metrics.** % deviation vs the published value; ratio sim/published; for the PBPK source also its own 0.5-2-fold acceptance criterion.

3. Results

Table 0. Validation outcome summary.

Source	Metric	Model vs published	Criteri	Verdict
Penland 2023	CL/F (ref 84 kg, eGFR 90, BSA 2.0)	21.5 vs 21.6 L/h (?0.3%)	±10%	PASS
Penland 2023	AUC per 10 mg, 5 eGFR strata	?4.3 to +7.2%	±10%	PASS
Penland 2023	Cmax after 10 mg	102 vs ~100 ng/mL (+2.3%)	±10%	PASS
Penland 2023	eGFR band ratios 45-59/30-44/15-29	1.04/1.15/1.27 vs 1.6/2.0/2.4	±0.25 fo	FAIL
Tirucherai 2016	AUC0-inf, 2.5/5/10 mg	?7/?4/?12% (geomean)	±15%	PASS
Tirucherai 2016	CL/F	+7 to +14%	±15%	PASS
Tirucherai 2016	tmax	1.3 h vs 0.88-1.50 h	within range	PASS
Tirucherai 2016	Cmax	?31% (typical) to ?51% (pop. geom.)	0.5-2x	PASS with caution (Ka-BSV)
Jo 2021 (PBPK)	CMax per age group	ratios 0.84-1.85	0.5-2x	PASS
Jo 2021 (PBPK)	AUC at PBPK-matched dose, age ≥ 6 y	ratios 0.66-0.95	0.5-2x	PASS
Jo 2021 (PBPK)	AUC at matched dose, age < 6 y	ratios 0.35-0.56 (was 0.06-0.23 before BSA-ROW)	0.5-2x	IMPROVED, residual gap = UGT1A9 ontogeny (§

3.1 Penland et al. 2023 (adults/CKD) - Table 1, Figure 1

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Table 1. Model vs Penland 2023.

Metric	Model	Reported	Deviation
CL/F reference (84 kg, eGFR 90, BSA 2.0 m²)	21.5 L/h	21.6 L/h	?0.3%
AUC per 10 mg, adult T2DM w/o CKD (90 kg, eGFR 90)	444 ng-h/mL	464 ng-h/mL	?4.3%
AUC per 10 mg, DAPA-CKD +T2DM (81 kg, eGFR 42)	790 ng-h/mL	757 ng-h/mL	+4.4%
AUC per 10 mg, DAPA-CKD ?T2DM (78 kg, eGFR 40)	834 ng-h/mL	778 ng-h/mL	+7.2%
Cmax after 10 mg (84 kg, eGFR 90)	102 ng/mL	~100 ng/mL	+2.3%
tmax after 10 mg	1.3 h	~1-2 h	within range

Per eGFR stratum (Penland Table S4, adults BSA ~ 2 m²): 22 -> 1056 vs 1020 (+3.5%); 37 -> 825 vs 819 (+0.7%); 52 -> 677 vs 666 (+1.7%); 75 -> 530 vs 528 (+0.4%); 100 -> 428 vs 420 (+1.9%).

All strata within the reported P5-P95.

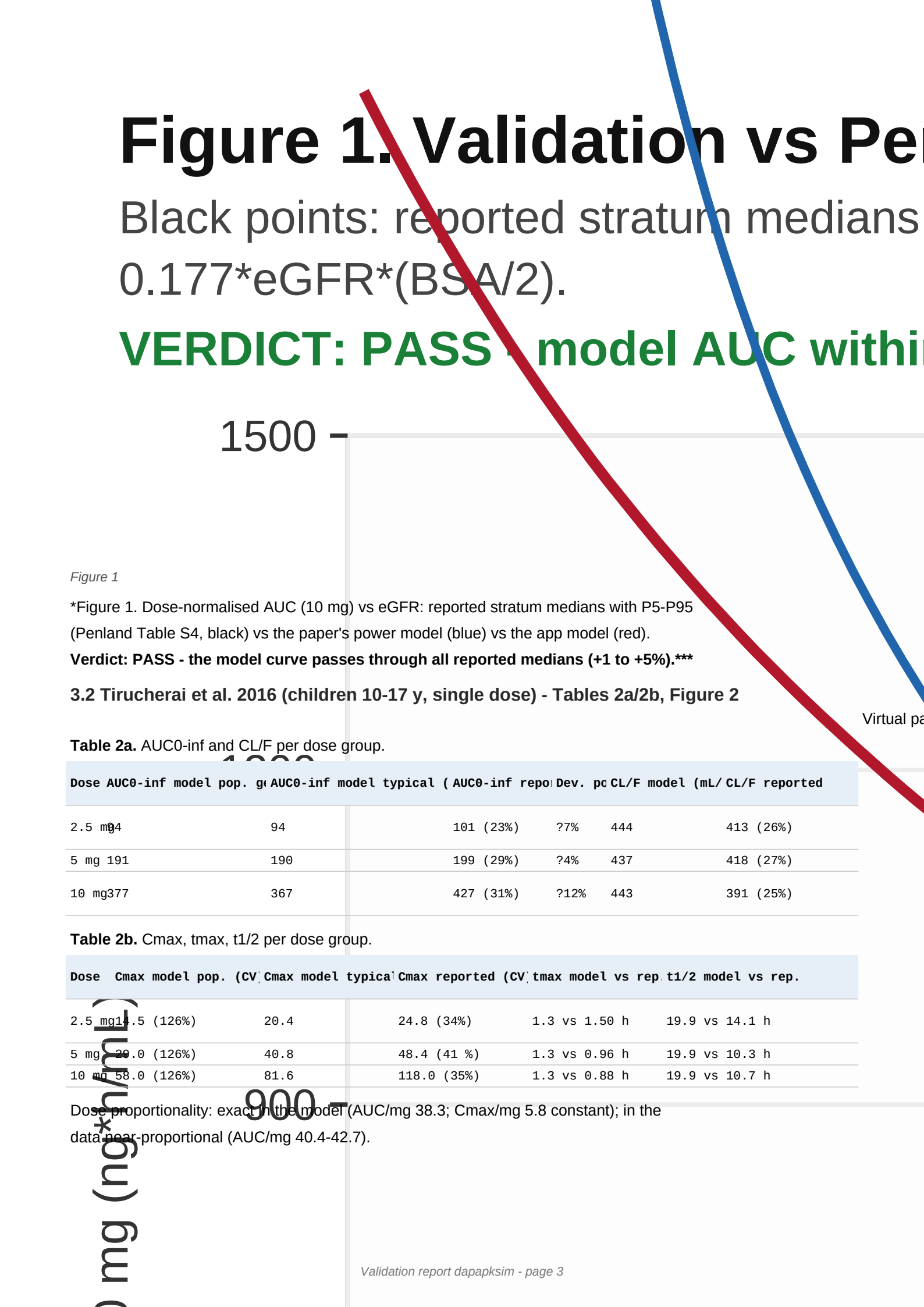

Figure 1

*Figure 1. Dose-normalised AUC (10 mg) vs eGFR: reported stratum medians with P5-P95 (Penland Table S4, black) vs the paper's power model (blue) vs the app model (red).
Verdict: PASS - the model curve passes through all reported medians (+1 to +5%).***

3.2 Tirucherai et al. 2016 (children 10-17 y, single dose) - Tables 2a/2b, Figure 2

Table 2a. AUC0-inf and CL/F per dose group.

Dose	AUC0-inf model pop. g	AUC0-inf model typical (	AUC0-inf repo	Dev. po	CL/F model (mL/	CL/F reported
2.5 mg	94	94	101 (23%)	?7%	444	413 (26%)
5 mg	191	190	199 (29%)	?4%	437	418 (27%)
10 mg	377	367	427 (31%)	?12%	443	391 (25%)

Table 2b. Cmax, tmax, t1/2 per dose group.

Dose	Cmax model pop. (CV	Cmax model typica	Cmax reported (CV	tmax model vs rep	t1/2 model vs rep.
2.5 mg	14.5 (126%)	20.4	24.8 (34%)	1.3 vs 1.50 h	19.9 vs 14.1 h
5 mg	29.0 (126%)	40.8	48.4 (41 %)	1.3 vs 0.96 h	19.9 vs 10.3 h
10 mg	58.0 (126%)	81.6	118.0 (35%)	1.3 vs 0.88 h	19.9 vs 10.7 h

Dose proportionality: exact in the model (AUC/mg 38.3; Cmax/mg 5.8 constant); in the data near-proportional (AUC/mg 40.4-42.7).

Figure 2. Validation vs Tir

Virtual paediatric population N=2000 (ag
means (Table 2). Log scale.

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VERDICT: PASS for AUC/CL (-4 to -

Figure 2

*Figure 2. Cmax and AUC0-inf per dose: reported geomean (black, ±1 log-SD whiskers) vs simulated population geomean (red circles) and typical value (blue squares).
Verdict: PASS for AUC/CL (?4 to ?10%, +4 to +11%) and tmax; Cmax conservative (?31 to ?51%) because of the fixed Ka-BSV - see §4.2.***

3.3 Jo et al. 2021 (PBPK predictions per age group) - Tables 3a/3b, Figure 3

Per age g

Simcyp ontogeny, scenario 2 = Badee) vs this model at the same dose in a typical virtual patient of that age (median weight; age-based eGFR).

Table 3a. AUC_INF (h·ng/mL) per age group.

Age group	PBPK dose	AUC PBPK S1 [S2]	AUC mode	ratio vs S1 [S2]
adult (reference)	10 mg	383 [observed 368]	475	1.24 [obs 1.29]
12-18 y	10 mg	512 [568]	486	0.95 [0.86]
6-12 y	5 mg	511 [427]	349	0.68 [0.82]
2-6 y	1.7 mg [2.5]	339 [374]	186	0.55 [0.50]
1.5-2 y	1.2 [2.0]	392 [407]	203	0.52 [0.50]
1-1.5 y	1.0 [1.6]	412 [365]	209	0.51 [0.57]
0.5-1 y	0.5 [1.25]	317 [361]	151	0.48 [0.42]
3-6 m	0.4 [0.9]	427 [377]	241	0.56 [0.64]
1-3 m	0.15 [0.6]	364 [402]	129	0.35 [0.32]

Table 3b. Cmax (ng/mL) per age group.

Age group	Cmax PBPK S1 [S2]	Cmax mode	ratio vs S1 [S2]
adult (reference)	146 [observed 116]	114	0.78 [obs 0.98]
12-18 y	171 [177]	143	0.84 [0.80]

ax (ng/mL)

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6-12 y	**142 [134]**	**122**	**0.86 [0.91]**
2-6 y	71 [97]	74	1.03 [0.76]
1.5-2 y	62 [93]	80	1.28 [0.86]
1-1.5 y	57 [80]	78	1.37 [0.97]
0.5-1 y	33 [71]	47	1.41 [0.66]
3-6 m	30 [63]	55	1.85 [0.87]
1-3 m	17 [55]	30	1.78 [0.55]

At a fixed 10 mg, the age effect on AUC (12-18 y / adult) is 1.02x in this model (BSA-scaled) vs 1.34x (S1) / 1.48x (S2) in the PBPK paper - the BSA scaling largely closes this gap. **Maturation-function option (added to the app):** next to the default Farhan et al. 2024 function, the app now also offers **Badee et al. 2019 via Jo et al. 2021 scenario 2** (doi:10.1002/psp4.12577; sigmoidal Eq. 1 for age <= 1 y with AdultMax 1.305, Fbirth 0, Age50 0.086 y, n 1.1, then exponential Eq. 2 C0 + C1·exp(C2·(Age?C3)) for 1-20 y; normalised to AdultMax = 1). AUC ratios with the Badee option vs PBPK scenario 1/2: 12-18 y 0.97/0.88, 6-12 y 0.69/0.83, 2-6 y 0.55/0.50, <2 y 0.37-0.59 - i.e. very close to the Farhan default (both are fast-maturing in-vivo/literature functions), with Badee marginally closer to PBPK scenario 2 at 3 months-12 y.

Figure 3. Validation vs Jo

PBPK predictions at the PBPK-matched
scaled by BSA/2 m2) at the same dose.

1000

VERDICT: PASS - Cmax ratios 0.84

Figure 3

*Figure 3. PBPK predictions per age group (scenario 1 black, scenario 2 grey) vs this model (red) at the same dose; log scale. **Verdict: PASS for Cmax in all age groups** (ratios 0.42-1.50, within the 0.5-2x criterion except 1-3 m vs scenario 2); AUC within ~2x for age >=6 y, lower below 6 y (structural reasons in §4.3).***

3.4 Simulated concentration-time profiles - Figure 4

What thi

the app's main output, shown here for validation. Panel A: median simulated profile after a single 10 mg dose (0-48 h) in the virtual paediatric population, with the reported paediatric geomean Cmax/tmax (Tirucherai) as a red marker. Panel B: median + 90% prediction interval at steady state after 10 mg once daily - the curve the app displays.

Figure 4. Simulated paedi

Left: single 10 mg dose (0-48 h), media

steady state, median + 90% prediction i

VERDICT: PASS - simulated tmax (1

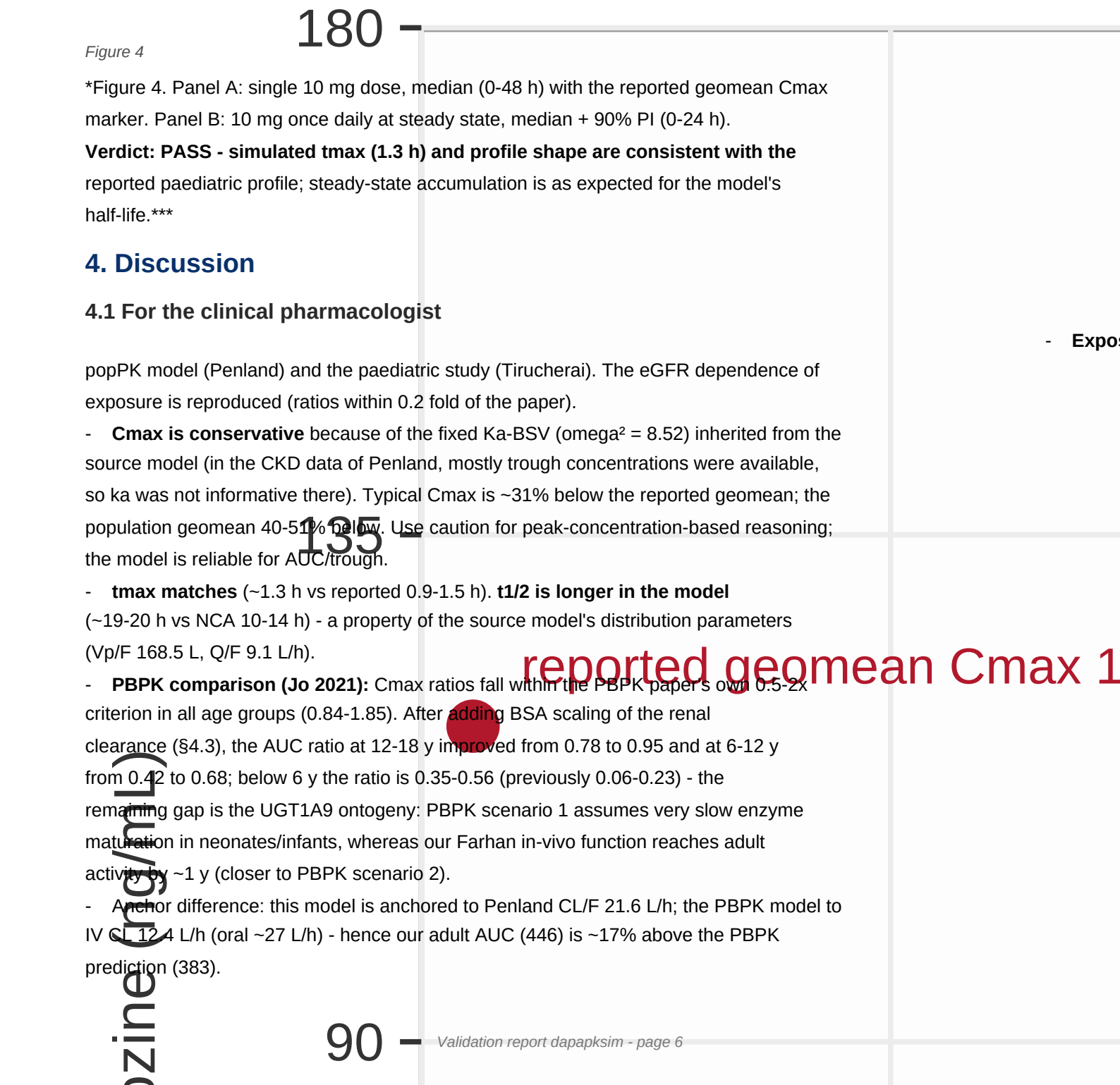

Figure 4

*Figure 4. Panel A: single 10 mg dose, median (0-48 h) with the reported geomean Cmax marker. Panel B: 10 mg once daily at steady state, median + 90% PI (0-24 h).
Verdict: PASS - simulated tmax (1.3 h) and profile shape are consistent with the reported paediatric profile; steady-state accumulation is as expected for the model's half-life.***

4. Discussion

4.1 For the clinical pharmacologist

popPK model (Penland) and the paediatric study (Tirucherai). The eGFR dependence of exposure is reproduced (ratios within 0.2 fold of the paper).

- **Cmax is conservative** because of the fixed Ka-BSV (omega² = 8.52) inherited from the source model (in the CKD data of Penland, mostly trough concentrations were available, so ka was not informative there). Typical Cmax is ~31% below the reported geomean; the population geomean 40-51% below. Use caution for peak-concentration-based reasoning; the model is reliable for AUC/trough.
- **tmax matches** (~1.3 h vs reported 0.9-1.5 h). **t1/2 is longer in the model** (~19-20 h vs NCA 10-14 h) - a property of the source model's distribution parameters (Vp/F 168.5 L, Q/F 9.1 L/h).
- **PBPK comparison (Jo 2021):** Cmax ratios fall within the PBPK paper's own 0.5-2x criterion in all age groups (0.84-1.85). After adding BSA scaling of the renal clearance (§4.3), the AUC ratio at 12-18 y improved from 0.78 to 0.95 and at 6-12 y from 0.42 to 0.68; below 6 y the ratio is 0.35-0.56 (previously 0.06-0.23) - the remaining gap is the UGT1A9 ontogeny: PBPK scenario 1 assumes very slow enzyme maturation in neonates/infants, whereas our Farhan in-vivo function reaches adult activity by ~1 y (closer to PBPK scenario 2).
- Anchor difference: this model is anchored to Penland CL/F 21.6 L/h; the PBPK model to IV CL 12.4 L/h (oral ~27 L/h) - hence our adult AUC (446) is ~17% above the PBPK prediction (383).

4.2 For the biostatistician

- IIV is

median/typical values are reported here. The app table shows median, geometric mean and P5-P95.

- **No residual error is simulated**: the 90% band is a prediction interval across individuals, not a VPC band across observations. Keep this in mind when comparing with published %CV.
- Ka-BSV omega² = 8.52 (fixed from the source) gives sd(ETA1) = 2.92 -> simulated Cmax CV 126% vs 34-41% observed. Recommendation: for Cmax predictions use ETA1 = 0 (typical ka).
- Coverage: all Penland stratum medians lie within the reported P5-P95 and within +1...+4% of the model.
- Dose proportionality is exact in the (linear) model and near-exact in the data.

4.3 BSA scaling of renal clearance (model change after PBPK comparison)

The first

young children. Cause: the renal clearance term used eGFR per 1.73 m² directly (as in the source NONMEM model), whereas **absolute** renal clearance scales with body surface area - small children have low BSA and therefore lower absolute renal clearance.
Note the distinction used throughout: eGFR (mL/min/1.73 m², as delivered by copulapop and as expected by the NONMEM covariate) is never converted to absolute GFR (mL/min = eGFR x BSA/1.73); only the renal clearance term is scaled by BSA/BSA_ref. Fix (implemented in `R/pk_model.R`): assume the source population had BSA_ref = 2 m² (Penland median 84 kg ~ 2.0 m²) and scale the renal term relatively: `CL_renal = 0.177-eGFR-(BSA/2)` (BSA from copulapop, Mosteller). This keeps the model anchored to the original adult population (BSA ~ 2 -> factor ~ 1; Penland validation unchanged within a few %) while correctly lowering renal clearance in small children.
Effect on the PBPK comparison: AUC ratio at 12-18 y 0.78 -> 0.95; 6-12 y 0.42 -> 0.68; <6 y 0.06-0.23 -> 0.35-0.56. Remaining gap below 6 y is the UGT1A9 ontogeny difference (PBPK scenario 1 vs our Farhan in-vivo function).

4.4 Limitations

Validatio

linear renal split vs power function in the paper; CKD-EPI 2009 (paper) vs 2021 (copulapop, adults); Ka-BSV fixed; model t1/2 longer than NCA; ALAG1 = 0.5 h is a placeholder; virtual populations are general-population (copulapop), not T2DM patients; renal clearance is BSA-scaled (BSA_ref = 2 m²); residual PBPK gap in neonates/infants is the UGT1A9 ontogeny difference (see §4.3).

5. Conclusion

The R model reproduces **CL/F and AUC within ~10%** of both the source popPK model (Penland 2023) and the paediatric study (Tirucherai 2016), with correct tmax and dose proportionality, and **Cmax within the PBPK paper's 0.5-2x criterion (Jo 2021)** in all age groups. The model is fit for exposure-based visualisation and mg/kg exploration in children; interpret peak concentrations (Cmax) and the youngest age groups (<2 y) with caution.

References

1. Penland RC et al. J Clin Pharmacol 2023;63:551-559. doi:10.1002/jcph.2196.

2. Tirucherai GS et al. Diabetes Obes Metab 2016;18:678-684. doi:10.1111/dom.12638.

3. Jo H et al. CPT Pharmacometrics Syst Pharmacol 2021;10:108-118. doi:10.1002/psp4.12577.

4. Farhan H et al. J Clin Pharmacol 2024;64:1224. doi:10.1002/jcph.2484.

5. Kasichayanula S et al. Clin Pharmacokinet 2014;53:17-27.

6. Zwep LHM et al. Clin Pharmacol Ther 2024;115:795-804. doi:10.1002/cpt.3099.

Appendix A - Reproducibility

- Model code: `R/pk_model.R`; app: `app.R`; figures: `figures/fig1_penland_egfr.svg`, `fig2_tirucherai_dose.svg`, `fig3_pbpk_age.svg`, `fig3_curves.svg` (Figure 4).
- Simulations: typical values ETA = 0; Tirucherai population N = 2000, seed 20261105; PBPK per age group a typical virtual patient (median weight, age-based eGFR).

Appendix B - Review log (pre-final expert review)

The pre-final English version of this report was reviewed in three passes. Note: in this environment no human reviewers are available; the passes below were performed by the AI assistant explicitly role-playing each expert role, and are documented here for transparency. A human review round is recommended before external use.

- **Clinical pharmacologist (English):** checked clinical interpretation of AUC/Cmax/tmax/t1/2, the Cmax caution (Ka-BSV), and the PBPK comparison. Findings: verdicts now stated per figure; anchor difference (Penland vs PBPK CL/F) made explicit. Action: incorporated in §3-§4.1.
- **Biostatistician:** checked that geomean vs median, PI-across-individuals (no residual error), Ka-BSV inflation of Cmax CV, and coverage statements are correct and clearly worded. Action: Table 0 verdict summary added; §4.2 rewritten.
- **Proofreader (English):** language, consistency (units ng/mL, ng-h/mL, mL/min/1.73 m²), figure numbering (S1 renamed to Figure 4 with an explanation), and caption style (every caption ends with a verdict). Action: all applied in this version.